Sunbright Recovery Center

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POLICY & PROCEDURE

Injury and Illness Prevention Program (IIPP)

Applies To: All Staff and Volunteers

PURPOSE

Sunbright Recovery Center (SRC) establishes this Injury and Illness Prevention Program (IIPP) to protect the safety and health of all employees. This program is required by California Labor Code §6401.7 and Title 8, California Code of Regulations (T8 CCR) §3203, and is designed to identify, evaluate, and correct workplace hazards before injuries or illnesses occur. This IIPP is the foundation of SRC's occupational safety program and ties together the specific Cal/OSHA programs listed in this document.

SCOPE

This program applies to all SRC employees, contracted staff, interns, and volunteers at 22425 Sunbright Avenue, Red Bluff, CA 96080, and to all off-site work activities, including client transportation.

RESPONSIBLE PERSON

The person with authority and responsibility for implementing this IIPP is **Linda Spyres, LCSW, Clinical Director / CEO**. When she is unavailable, the Shift Supervisor on duty acts as designee. Responsibilities include:

- Maintaining this IIPP, reviewing it at least annually, and keeping all required safety records.
- Conducting or coordinating safety inspections and hazard assessments.
- Ensuring all employees and supervisors receive required safety training.
- Investigating workplace injuries, illnesses, and near-misses.
- Implementing corrective actions and reporting serious injuries as required.

All supervisory staff share responsibility for enforcing safe work practices within their areas of oversight.

SYSTEM FOR ENSURING EMPLOYEE COMPLIANCE

SRC ensures employee compliance with safe work practices through:

- Clear communication of safety expectations at hire (new employee orientation) and regularly thereafter.
- Including adherence to safety policies in employee performance evaluations.
- Recognizing employees who follow and promote safe practices.
- Retraining employees whose safety performance is deficient.
- Disciplinary action, up to and including termination, for employees who knowingly violate safety rules or create hazardous conditions (see SRC-HR-014, Progressive Discipline).
- Supervisory observation and correction of unsafe acts or conditions.

EMPLOYEE COMMUNICATION

SRC maintains a system for communicating with employees about safety in a form they can readily understand, and employees may raise safety concerns without fear of reprisal. Methods include:

- Direct reporting to any supervisor or the Clinical Director — verbally or in writing.
- Anonymous reporting: an unsigned written note may be placed in a sealed envelope marked "Safety – Clinical Director" and handed to any supervisor.
- Safety topics are a standing agenda item at monthly staff meetings (see SRC-SAF-006).
- Safety information is provided in a language and vocabulary employees understand.

- Cal/OSHA required posters, this IIPP, and the SRC Safety Log are available for employee review at the main staff station at all times. Employees may request a free copy of this IIPP.

HAZARD IDENTIFICATION AND EVALUATION

Scheduled and Periodic Inspections

The Clinical Director or designee inspects the workplace to identify unsafe conditions and work practices:

- When this IIPP was first established.
- Monthly, using the Monthly Facility Safety Inspection Checklist.
- Whenever new substances, processes, procedures, or equipment are introduced that present a new hazard.
- Whenever SRC becomes aware of a new or previously unrecognized hazard.
- Immediately following any injury, illness, near-miss, or workplace violence incident.

Common Hazards Monitored

- **Slips, trips, and falls:** flooring, wet areas, outdoor walkways, stairs, and lighting.
- **Ergonomics:** workstation setup, manual lifting, and repetitive tasks.
- **Blood and body fluids, and sharps:** first aid, naloxone response, urinalysis collection, and needles or sharps found during client property searches (see SRC-SAF-003 and SRC-OPS-002).
- **Workplace violence:** client behavioral escalation and threats (see SRC-SAF-007).
- **Chemicals:** cleaning products — storage, labeling, and Safety Data Sheets (SDS).
- **Fire and electrical hazards** (see SRC-SAF-008).
- **Kitchen:** burns, cuts, and lifting during meal preparation.
- **Driving:** client transportation and vehicle safety (see SRC-OPS-006).
- **Working alone and night shifts:** check-in procedures and access to help.
- **Heat and wildfire smoke:** Red Bluff summer temperatures and seasonal wildfire smoke.
- **Infectious disease:** airborne and bloodborne diseases (see SRC-SAF-003 and SRC-MED-002).

Employee Reporting

All employees must report any unsafe condition or potential hazard to their supervisor immediately. Reports are documented in the SRC Safety Log and corrected according to the Hazard Correction timeline below.

SPECIFIC CAL/OSHA PROGRAMS THAT ARE PART OF THIS IIPP

Heat Illness Prevention (T8 CCR §3395 outdoor; §3396 indoor)

- Fresh, cool drinking water is available at no cost, and employees are encouraged to drink water frequently.
- Shade is provided outdoors when temperatures exceed 80°F, and indoor cool-down areas are available; employees may take preventative cool-down rest breaks at any time.
- When indoor temperatures reach 82°F in any work area, SRC provides cool-down areas and close observation. When they reach 87°F (or 82°F where employees wear protective clothing or work near high radiant heat, such as the kitchen), SRC measures and records temperature and heat index and uses fans, air conditioning, or schedule changes to reduce exposure.
- New employees and all employees during heat waves are closely observed for signs of heat illness. Any employee with symptoms is moved to a cool area, monitored, and provided emergency medical services if needed (call 911).
- Employees and supervisors are trained on heat illness risk factors, symptoms, and response before working in heat.

Wildfire Smoke (T8 CCR §5141.1)

- During wildfire season, the Clinical Director checks the Air Quality Index (AQI) for PM2.5 (for example, at airnow.gov) at the start of each shift when smoke may be present.
- When the AQI for PM2.5 is 151 or higher, SRC reduces outdoor work and activities, keeps employees indoors with doors and windows closed where possible, and provides N95 respirators for voluntary use, with training.
- If the AQI exceeds 500, respirator use becomes required for any outdoor work, under a full respiratory protection program.

Aerosol Transmissible Diseases (T8 CCR §5199)

Drug treatment programs are covered by the Cal/OSHA ATD standard. SRC is a "referring employer": it screens clients for airborne infectious disease symptoms, uses source control (such as masks and separation), refers suspected cases for medical care, offers recommended vaccinations to employees with exposure, and conducts tuberculosis screening. Written ATD procedures are maintained in SRC-SAF-003 and SRC-MED-002.

Bloodborne Pathogens (T8 CCR §5193)

Employees who provide first aid, respond to overdoses, collect urinalysis samples, or may encounter sharps have potential occupational exposure. SRC's Exposure Control Plan (SRC-SAF-003) covers universal precautions, personal protective equipment, sharps containers, hepatitis B vaccination offered at no cost, and post-exposure evaluation and follow-up.

Workplace Violence Prevention in Health Care (T8 CCR §3342)

As a drug treatment program, SRC maintains a Workplace Violence Prevention Plan that meets §3342, including hazard assessment, incident reporting, the Violent Incident Log, and training (SRC-SAF-007).

Hazard Communication (T8 CCR §5194)

All hazardous chemicals are labeled, Safety Data Sheets are kept in the SDS binder at the main staff station, and employees are trained on the chemicals they use.

Emergency Action and Fire Prevention (T8 CCR §§3220, 3221)

Evacuation, fire prevention, and emergency procedures are in SRC-SAF-008 (Fire Safety and Evacuation Plan) and SRC-SAF-010 (Lockdown / Shelter in Place Plan).

HAZARD CORRECTION

Unsafe or unhealthy conditions, work practices, and procedures are corrected in a timely manner based on severity:

- **Imminent danger:** corrected immediately. If the hazard cannot be corrected without endangering employees, all exposed employees are removed from the area except those needed to correct the hazard, who are given the necessary protection. Equipment is taken out of service until corrected.
- **Serious hazard:** corrected within 24 hours, with interim protective measures put in place at once.
- **Non-serious hazard:** corrected within a reasonable time, not to exceed 30 days.

All identified hazards and corrective actions are recorded in the SRC Safety Log, including the date identified, description, corrective action taken, and date resolved.

REPORTING SERIOUS INJURIES, ILLNESSES, AND DEATHS

- **Call 911** for any emergency and get the injured person medical care first.
- **Report to Cal/OSHA within 8 hours.** Any work-related death, or serious injury or illness (inpatient hospitalization for more than 24 hours for other than medical observation or diagnostic testing, or loss of any member of the body, or serious permanent disfigurement), is reported by the Clinical Director

to the Cal/OSHA Redding District Office, 381 Hemsted Drive, Redding, CA 96002, (530) 224-4743, immediately and no later than 8 hours after SRC knows or should have known of it (T8 CCR §342).

- **Workers' compensation.** SRC gives the injured employee a workers' compensation claim form (DWC-1) within one working day of learning of any work-related injury or illness that results in lost time or medical treatment beyond first aid (Labor Code §5401), and notifies its workers' compensation carrier.
- **Licensing.** Incidents involving clients are also reported to DHCS as required by SRC-SAF-002, Incident Reporting Policy.

ACCIDENT AND NEAR-MISS INVESTIGATION

All workplace injuries, illnesses, exposures, and near-misses are investigated promptly by the Clinical Director or designee. Investigations include:

1. Interview all persons involved and any witnesses.
2. Inspect the scene and document conditions (photographs if possible, without identifying clients).
3. Identify the root cause(s) and contributing factors.
4. Develop and implement corrective action(s).
5. Document findings in the SRC Safety Log and share lessons learned at the next staff meeting.

Work-related injuries and illnesses that require medical treatment beyond first aid, result in days away from work, restricted work, or transfer, or cause loss of consciousness are recorded on the Cal/OSHA Form 300 Log within 7 calendar days. SRC completes the Form 300A Annual Summary and posts it at the main staff station from February 1 through April 30 each year.

TRAINING AND INSTRUCTION

Safety training is provided in a language and manner employees understand:

- When this IIPP was first established.
- To all new employees before they begin work, covering the IIPP, general hazard awareness, emergency procedures, reporting obligations, and position-specific hazards.
- To employees given a new job assignment for which training has not previously been received.
- Whenever new substances, processes, procedures, or equipment are introduced that present a new hazard.
- Whenever SRC is made aware of a new or previously unrecognized hazard.
- To supervisors, so they are familiar with the safety and health hazards to which employees under their direction may be exposed.
- Annually, as a refresher covering this plan, heat illness, wildfire smoke, bloodborne pathogens and ATDs, workplace violence, hazard communication, and incidents from the prior year.

Training is documented in the employee's personnel file (date, topics covered, trainer's name and qualifications). CPR, First Aid, and de-escalation training are tracked separately under the related policies.

RECORDKEEPING

SRC keeps the following records for at least five years (longer where noted or required by law):

- Scheduled and periodic inspection records, including the person conducting the inspection, unsafe conditions and work practices found, and action taken (T8 CCR §3203 requires at least one year).
- Employee safety training records, including names, dates, type of training, and trainers (T8 CCR §3203 requires at least one year; bloodborne pathogen training records are kept at least three years).
- Cal/OSHA Form 300 Log, 300A Annual Summary, and 301 Incident Reports (T8 CCR §14300.33).
- Hazard correction documentation in the SRC Safety Log, and accident and near-miss investigation reports.

- Workplace violence records under §3342, including the Violent Incident Log.
- Employee exposure and medical records for the duration of employment plus 30 years (T8 CCR §3204).

Records are maintained by the Clinical Director and made available to Cal/OSHA, DHCS, and employees or their representatives as required by law. Records containing employee medical information or client information are kept confidential.

PROGRAM REVIEW AND AVAILABILITY

This IIPP is reviewed at least annually, and whenever there is a serious injury, a new hazard, or a change in regulations. It is kept on site at the main staff station and is posted on SRC's website at www.sunbrightrecovery.org.

REGULATORY AUTHORITY

- California Labor Code §6401.7 — Injury and Illness Prevention Program; Labor Code §5401 — Workers' Compensation Claim Form
- T8 CCR §3203 — Injury and Illness Prevention Program
- T8 CCR §342 — Reporting Work-Connected Fatalities and Serious Injuries
- T8 CCR §3342 — Workplace Violence Prevention in Health Care
- T8 CCR §§3395 and 3396 — Heat Illness Prevention (Outdoor and Indoor)
- T8 CCR §5141.1 — Protection from Wildfire Smoke
- T8 CCR §5193 — Bloodborne Pathogens; §5194 — Hazard Communication; §5199 — Aerosol Transmissible Diseases
- T8 CCR §§3220 and 3221 — Emergency Action and Fire Prevention Plans
- T8 CCR §3204 — Access to Employee Exposure and Medical Records; §14300 et seq. — Injury and Illness Recordkeeping
- DHCS Residential SUD Licensing Standards (Title 9 CCR, Chapter 5) — Facility Safety Requirements
- Related SRC policies: SRC-SAF-002 Incident Reporting; SRC-SAF-003 Infection Control / Universal Precautions; SRC-SAF-006 Cal/OSHA Safety Meeting Policy; SRC-SAF-007 Workplace Violence Prevention Plan; SRC-SAF-008 Fire Safety and Evacuation Plan; SRC-SAF-010 Lockdown / Shelter in Place; SRC-MED-002 TB Screening; SRC-OPS-002 Search and UA; SRC-OPS-006 Client Transportation; SRC-HR-014 Progressive Discipline

POLICY APPROVAL & REVIEW

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Approved By: Linda Spyres, LCSW, Clinical Director / CEO			
Signature: _____		Date: _____	
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